

ADDRESS TO THE SPEAKER
OF THE NATIONAL ASSEMBLY
PARLIAMENT OF THE REPUBLIC OF SOUTH AFRICA

CONSOLIDATED FORENSIC CLOSURE REPORT
Regulatory Capture, Actuarial Fraud & Whistleblower Attrition

Entity	Role	Statutory Mandate
Council for Medical Schemes (CMS)	Statutory Regulator	Medical Schemes Act 131 of 1998 – Sections 7, 44, 47, 58
Government Employees Medical Scheme (GEMS)	Open Medical Scheme	1.9 million beneficiaries – State employee compensation pool
Medscheme Holdings (AfroCentric Group)	Accredited Administrator	Medical Schemes Act – Administrator accreditation obligations

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1. OBJECTIVE OF THIS DOCUMENT

This consolidated forensic closure report is addressed directly to the Honourable Speaker of the National Assembly and to the relevant Portfolio Committees of Parliament. It is structurally positioned to provide the Portfolio Committee on Health and parliamentary oversight mechanisms with immediate visibility into an active, ongoing conspiracy of regulatory capture.

This document serves as a diagnostic bridge exposing how systemic software integration failures are leveraged to extract illicit capital from consumers, and how statutory regulators have actively weaponised the legal system to silence protected disclosures.

Rather than a historical archive, this document exposes live collusion occurring between the Council for Medical Schemes (CMS), Government Employees Medical Scheme (GEMS), and Medscheme Holdings (AfroCentric Group). The record in the possession of the CMS is complete. The technical Root Cause Analysis is unassailable.

This report demands immediate parliamentary intervention to compel a Section 44 forensic inspection into the corrupted IT architecture of the accredited administrator, and to issue binding subpoenas for the communication logs proving multi-agency collusion.

2. THE PRIMA FACIE CASE: PUBLIC FINANCE VS. CORPORATE PRIVILEGE

This briefing establishes a clear prima facie case of institutional collusion and regulatory capture between the CMS, GEMS, and Medscheme Holdings. There is a profound, documented body of evidence showing that the statutory watchdog has actively operated in tandem with the regulated entities as a corporate cartel, prioritising corporate secrecy over its explicit mandate to protect 1.9 million beneficiaries.

FOUNDATIONAL LEGAL PREMISE: Confidentiality and proprietary privilege are not above public finances, statutory accountability, or consumer protection. Corporate entities cannot utilise High Court litigation or narrow intellectual property claims to hide systemic software integration defects that siphon multi-billion-rand 'Governance Taxes' from public funds and state employee compensation pools.

The evidence already in hand is more than sufficient to compel an immediate, uncompromised uncovering of internal corporate communications.

3. THE APN RISK LOADING FRAUD – THE “GOVERNANCE TAX” CALCULUS

A baseline clinical risk variance within large-scale medical schemes typically operates within a standard 2% to 4% margin. However, GEMS and its administrator Medscheme implemented an aggressive 9.8% APN Risk Loading model. To legally sustain this inflated premium extraction under the Medical Schemes Act 131 of 1998, the entities were statutorily required to submit an empirically backed business plan to the CMS justifying the inflation based on beneficiary health risk profiles.

The CMS approved this business plan and sanctioned the resulting 9.8% to 9.5% “Governance Tax” despite possessing direct knowledge that the underlying data framework was entirely corrupted.

3.1 The Data Corruption Mechanics

Defect	Mechanism
The Structural Blind Spot	A verified Root Cause Analysis (RCA) proved that a hardcoded architectural design flaw – the Gexus-DMS Integration Gap – permanently blinded the Disease Management System (DMS) whenever a chronic patient transitioned from acute to chronic benefits.
The Telemetry Failure	While the Gexus claims system recorded valid, compliant chronic claims, the tracking system (DMS) failed to receive the data loop, automatically flagging fully compliant beneficiaries as “non-adherent”.
Artificial Risk Inflation	By accumulating thousands of false non-adherence profiles, the administrator fabricated a synthetic crisis of chronic patient delinquency. This artificial inflation of scheme risk was utilised to justify the premium loading.
Imminent Realignment	This systemic extraction continues until the 1 July 2026 premium adjustments, where an artificial decrease is being engineered to quietly offload the toxic data trail before executive audit intervention.

4. THE PARLIAMENTARY SUBPOENA MANDATE

To prevent the CMS from continuing to disregard the safety and financial security of its membership base, Parliament must exercise its statutory oversight powers to issue explicit, legally binding subpoenas for the following three sets of unredacted records:

4.1 The GEMS Actuarial Business Plans

- Parliament must subpoena the original, unredacted business plans and empirical risk-modelling data submitted by GEMS to the CMS to legally justify the initial 9.8% APN Risk Loading model.

- This submission must be forensically cross-referenced against the internal IT architecture to prove that the CMS approved these inflated premium extractions while fully aware that the data was derived from a corrupted tracking loop.

4.2 Coordinated Communication Logs – The GEMS-CMS-Medscheme Triad

- A comprehensive subpoena must compel the immediate release of all email correspondence, meeting minutes, and telephonic telemetry between GEMS, CMS, and Medscheme Holdings.
- The search parameters must target the critical operational windows surrounding the litigation timeline: specifically, the days leading up to and immediately following the High Court Interim Order of 16 April 2026, stretching through to the coordinated actions of 7 May 2026.

4.3 Explicit Whistleblower Targeting Logs

- The communication subpoena must mandate a strict keyword audit across all internal servers of the CMS, GEMS, and Medscheme using the specific search terms: “Wilbur William Matthee”, “Matthee”, and “EthicHawks”.
- Unmasking these internal discussions is the only definitive mechanism required to justify the allegations of active, multi-agency collusion, proving that the regulatory watchdog deliberately coordinated its legal strategy with the corporate respondents to penalise and silence the specialist investigator.

5. THE MAY 7TH COINCIDENCE – THE ACTUARIAL RETREAT AND THE LEGAL TRAP

The absolute necessity of these subpoenas is demonstrated by the synchronised corporate and regulatory actions that took place on 7 May 2026.

THE ACTUARIAL RETREAT	THE REGULATORY TRAP
GEMS formally announced a sudden reduction of the premium loading figure down to 7.5%, effective 1 July 2026, to shrink the audit trail.	CMS Senior Legal Adjudication Officer Lebogang Mpumlwana issued a directive demanding granular data, attempting to bait the whistleblower into a contempt of court arrest.

5.1 The Admission by Conduct

On 7 May 2026, GEMS formally announced that it would reduce its premium loading figure down to 7.5%, effective 1 July 2026. This sudden reduction serves as a direct admission by conduct. Because the underlying Gexus-DMS Integration Gap software defect remains uncorrected, this arbitrary adjustment proves that the original 9.8% loading was never based on legitimate clinical health risks. It was a synthetic calculation that the institutions is now scrambling to lower in order to reduce the financial footprint of the fraud before parliamentary appropriations committees conduct a line-item intervention.

5.2 The Coordinated Silence Strategy – Weaponising Contempt of Court

Simultaneously, on 7 May 2026 at 12:01 PM, Senior Legal Adjudication Officer Lebogang Mpumlwana sent a directive demanding that the whistleblower fill out new operational complaint forms. This was a deliberate legal trap executed on the exact same day as the scheme's premium adjustment.

The CMS's complicity shifted from passive negligence to active, criminal collusion following the granting of the High Court Interim Order (Case No: 2026-086906) on 16 April 2026. The mechanics of the trap operated as follows:

Step	Action
16 April 2026	High Court Interim Order issued – gagging disclosure of operational systems under judicial review.
7 May 2026, 12:01	CMS directs whistleblower to file new, highly granular complaint form identifying parties, specific acts, and internal operational evidence.
If Whistleblower Complies	Immediate contempt of court arrest: CMS would pass restricted data to corporate respondents for “administrative comment,” triggering breach of the High Court order.
If Whistleblower Refuses	CMS closes file for “non-compliance,” cloaking regulatory inaction behind the appearance of whistleblower obstruction.

This manoeuvre was formally executed on 7 May 2026 to neutralise the High Court evidence bundle detailing the systemic APN risk loading fraud. The CMS issued this demand despite having a complete, exhaustive evidentiary record and sworn affidavit files in their possession since 23 December 2025 and 20 January 2026.

6. EXECUTIVE COMPLICITY & PERJURY – THE COMPANY SECRETARY'S OATH

The architectural cover-up of the Gexus-DMS integration gap was actively shielded at the highest executive level. This complicity is anchored by the actions and sworn statements of Company Secretary Lebohlang Mpumlwana.

As the statutory gatekeeper of the board, the Company Secretary holds a fiduciary duty to ensure the board is fully apprised of material legal and operational risks. Instead, Mpumlwana weaponised her position to suppress the forensic disclosures.

Act of Complicity	Detail
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9 December P1 Risk Report Suppression	Mpumlwana was a primary recipient of the initial Priority 1 (P1) Risk Report distributed on 9 December 2025. Her sole acknowledgment was a deflection, merely informing the whistleblower that a specific risk committee member (Mr. Arke) was no longer on the board.
High Court Contradiction	Mpumlwana explicitly stated under oath in the High Court that the Deputy Information Officer claimed the integration gap “did not happen.”
The Logical Impossibility	The internal systemic Root Cause Analysis (RCA) is definitively documented on the administrator’s own internal servers. Either the Deputy Information Officer was actively lied to by operational executives, or the Deputy Information Officer is complicit in the cover-up.

By deliberately suppressing the 9 December P1 Risk Report and subsequently facilitating contradictory sworn testimony in the High Court, the Company Secretary failed in her fiduciary duties. She is fundamentally complicit in the institutions operations, cementing her status as a primary target in the active Section 162 Director Delinquency Application.

7. CHRONOLOGY OF REGULATORY EVASION & JURISDICTIONAL PING-PONG

The institution’s systemic methodology of structural attrition and shifting goalposts is preserved through an unbroken chain of primary legal files, which Parliament must review alongside the subpoenaed logs:

Date	File Reference	Forensic Significance
23 December 2025	CMS INITIAL DISCLOSURE.PDF	Primary evidence bundle and sworn affidavit detailing administrative misconduct, unlawful deductions, and the executive suppression of a Protected Disclosure.
20 January 2026	CMS COMPLAINT FORM.PDF	Formal complaint submitted under Section 47(1) of the MSA, explicitly identifying GEMS Member 000655789 and detailing how Team Leader Mary-Ann Jones forced staff to utilise a corrupted system architecture to manipulate patient adherence profiles.
4 February 2026	CMS Jurisdictional Declination	CMS falsely categorises a multi-million-rand structural system failure as a localised ‘labour/employment dispute’ and unprocedurally shifts the file to the HPCSA.
16 February 2026	CMS 16 Feb 2026.PDF	Whistleblower completes the entire statutory referral chain (CMS → HPCSA → SANC → SAPC), proving that all regulators engaged in jurisdictional ping-pong to avoid inspecting the core infrastructure defect.
2 April 2026	CMS REENGAGE FOLLOW UP.PDF	Formal Jurisdictional Re-Engagement Notice served on the CMS. Disclosure reframed under Sections 7, 44, 47, and 58 of the MSA, citing the 2026 Discovery Health pharmacy

		overpayments case as precedent for CMS jurisdiction over software failures.
7 May 2026, 12:01	CMS 7 May 2026.PDF	Formal directive from Lebogang Mokalake attempting to trigger non-compliance or a breach of the High Court Interim Order.
7 May 2026, 17:03	CMS Final Notice 7 May 26.PDF	Whistleblower's formal legal response: record complete, High Court order attached, any further evasion constitutes actionable dereliction of statutory duty.

8. ATTRITION BY MANUFACTURED DESTITUTION

This pre-trial record establishes that the institutions primary defence mechanism is the physical and economic elimination of the whistleblower.

As a direct consequence of refusing to execute professionally negligent instructions based on corrupted data, the specialist nurse was subjected to:

- An unlawful 40% salary deduction, in breach of Section 34 of the BCEA.
- Blockage from professional registration with the SANC.
- Victimisation constituting automatically unfair dismissal under Section 187(1)(h) of the LRA.
- Forced into absolute homelessness and destitution.

EVIDENTIARY RECORD OF DESTITUTION: The whistleblower's official residential address on the standard CMS complaint registry stands recorded as: "None/destitute as a result of employer Medscheme conduct."

The CMS chose to validate this corporate retaliation by dragging out investigations across multiple claims cycles and attempting to coordinate legal traps. By stretching out investigations, ignoring an active system failure affecting 1.9 million beneficiaries, and attempting to coordinate legal traps with corporate respondents, the CMS has actively validated corporate retaliation.

They have consciously relied on poverty and structural silencing to ensure that the whistleblower is broken before these subpoenaed communication logs can be brought to light on the floor of Parliament.

9. CONCLUDING MANDATE TO PARLIAMENT

The record in the possession of the CMS is complete. The technical Root Cause Analysis is unassailable. This consolidated report demonstrates that the regulatory framework in South Africa is no longer protecting the public interest; it is an active joint venture shielding corporate

administrators while weaponising state machinery to eliminate state-registered professionals who refuse to compromise patient data integrity.

Parliament is respectfully called upon to exercise the following powers without further delay:

#	Demand	Legal Basis
1	Immediately issue subpoenas for the three evidence sets identified in Section 4 of this report	Sections 55(2) & 56 of the Constitution; MSA Section 44
2	Compel a Section 44 forensic inspection into the Gexus-DMS Integration Gap and the corrupted IT architecture of the accredited administrator	Medical Schemes Act 131 of 1998, Section 44
3	Investigate the synchronised 7 May 2026 actions of GEMS and the CMS as evidence of active cartel conduct	Competition Act; MSA Section 47; PRECCA
4	Investigate the High Court sworn statements of Company Secretary Lebohang Khumbolwana for potential perjury	Criminal Procedure Act; Companies Act Section 162
5	Refer the CMS's jurisdictional ping-pong conduct to the Public Protector for investigation as maladministration	Public Protector Act; PAJA
6	Direct an immediate halt to the 1 July 2026 APN premium adjustment pending forensic audit of the underlying risk model	MSA Section 29 – prescribed minimum benefits; Section 44

**THE RECORD IS COMPLETE. THE EVIDENCE IS IMMUTABLE. THE
ATTRITION ENDS HERE.**

10. CERTIFICATE OF SERVICE

I, Wilbur William Matthee, hereby certify that a true copy of this Consolidated Forensic Closure Report and Parliamentary Address has been transmitted to the following recipients:

Recipient	Mode of Service
The Honourable Speaker of the National Assembly	Formal email and portfolio committee channels
Portfolio Committee on Health – Chairperson	vramaano@parliament.gov.za
Portfolio Committee on Justice and Constitutional Development	mmthonjeni@parliament.gov.za
Portfolio Committee on Employment and Labour	Parliamentary channels
Standing Committee on Finance	Parliamentary channels

Council for Medical Schemes – Chief Executive Officer	Registered electronic service
Government Employees Medical Scheme – Principal Officer	Registered electronic service
Medscheme Holdings – Chief Executive Officer (AfroCentric)	Via Deneys Reitz Inc. (Jason Whyte / Frances Barker)
South African Human Rights Commission	info@sahrc.org.za
Information Regulator of South Africa	Formal complaint to be filed separately
Office of the Chief Justice – Director David	RDavid@judiciary.org.za
Legal Practice Council	To be submitted separately

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Enclosures

- CMS Initial Disclosure.PDF (23 December 2025) – Primary sworn affidavit and evidence bundle
- CMS Complaint Form.PDF (20 January 2026) – Section 47(1) formal complaint
- CMS 16 Feb 2026.PDF – Jurisdictional referral chain completion
- CMS Reengage Follow Up.PDF / CMS_FollowUp_EthicHawks_2April2026.pdf (2 April 2026)
- CMS 7 May 2026.PDF – Mokalake contempt trap directive (12:01)
- CMS Final Notice 7 May 26.PDF – Whistleblower's formal legal response (17:03)
- High Court Case 2026-086906 – Interim Order of 16 April 2026
- GEMS APN Premium Loading Adjustment Notice (7 May 2026 – reduction to 7.5%)
- Root Cause Analysis (RCA) – Gexus-DMS Integration Gap
- P1 Risk Report (9 December 2025) – Suppressed by Company Secretary Khumbolwana
- Consolidated Forensic Oversight Report (Full Digital Dossier – SHA-256 verified)
- EthicHawks_Final_Closure_Report_OCJ_23June2026.docx – OCJ Judicial Integrity Report

END OF REPORT